

SKIN & LASER

Fitzpatrick Assessment & Test Patching

The assessment that every other protocol in this kit depends on

DOCUMENT SKN-007	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Every parameter decision in this kit is anchored to skin type. This protocol standardizes how Fitzpatrick type is assessed and recorded, how baseline photography is captured, and how test patches are performed, timed and interpreted.

ASSESS, DO NOT GUESS

Skin type is determined by how skin behaves in sunlight, not by how it looks across a consultation room. Two patients with similar apparent complexion can burn and tan very differently. Ask the questions, score them, and record the score.

2 Fitzpatrick Assessment

Administer the practice's standardized questionnaire at intake and record the resulting type in a fixed field in the chart, not in free text where it will be missed.

Type	Sun response	Practice implication
I	Always burns, never tans.	Highest efficacy across devices; lowest pigmentary risk. Watch for erythema-prone response.
II	Usually burns, tans minimally.	Generally straightforward across modalities.
III	Sometimes burns, tans uniformly.	PIH risk begins to matter. Prime for peels and resurfacing.
IV	Rarely burns, tans easily.	IPL excluded for hair reduction. Longer wavelengths, longer pulses, lower fluence.
V	Very rarely burns, tans profusely.	1064 nm for hair reduction. Conservative everything. Extended test patch intervals.
VI	Never burns, deeply pigmented.	Highest pigmentary risk. Most conservative parameters. Some services declined or referred.

Factors that modify the assessment

- Current tan — a tanned Fitzpatrick II behaves like a higher type. Treat to the current state, not the baseline.
- Ethnic background and family history of pigmentary response.
- History of PIH after acne, injury, waxing or prior treatment — a strong predictor and more useful than the score alone.
- History of melasma.

- Seasonal variation — reassess at every visit rather than relying on the intake score.

REASSESS AT EVERY SESSION

Skin type is recorded once; tanning status is checked every single time. A patient treated safely in February and burned in July was not a parameter failure — it was a screening failure.

3 Photography Standards

- ☐ Practice-owned device only. Never a personal phone. See OPS-005.
- ☐ Fixed lighting, fixed distance, fixed background, consistent camera settings.
- ☐ Standard angles per treatment area, replicated at every review.
- ☐ Makeup removed, hair secured, jewelry removed.
- ☐ No filters, no retouching, no beautification modes — many phones apply these by default and they must be disabled.
- ☐ Timestamped and stored in the clinical record system, not a camera roll.
- ☐ Baseline before the first treatment, then at defined intervals and at any complication.
- ☐ Clinical photography consent obtained separately from marketing consent, per OPS-002 and OPS-005.

PHOTOGRAPHS WORK BOTH WAYS

Standardized before images protect the patient and the practice equally. They evidence the result you achieved, and they evidence what was already present before you treated — including pigmentation or scarring the patient later attributes to you.

4 Test Patching

A test patch predicts the delayed response, which is the response that causes harm. Immediate observation alone is insufficient.

Situation	Test patch required
New patient, any energy-based device	Yes.
Existing patient, new device or platform	Yes — this is a new treatment.
Existing patient, new treatment area	Yes.
Existing patient, material parameter increase	Yes.
Fitzpatrick IV–VI, any energy-based treatment	Yes, with an extended review interval.
Recent sun exposure, tanning or self-tanner	Do not test or treat until the exclusion window has passed.
Medium-depth peel or resurfacing	A test area is used, sited discreetly.
Routine repeat session, same device, same area, stable parameters	No.

- 1

Site the patch within or immediately adjacent to the intended treatment area so the skin is representative — not on the forearm for a facial treatment.

- 2

Use the intended parameters. A conservative test tells you nothing about the treatment you plan to deliver.
- 3

Photograph the site before and immediately after.
- 4

Observe the immediate response and record it.
- 5

Wait the review interval set by your medical director — longer for Fitzpatrick IV–VI, since PIH is delayed.
- 6

Review in person and photograph again. A patient-reported review by text is not adequate for a first patch in higher skin types.
- 7

Record the go / no-go decision and any parameter adjustment made as a result.

Review finding	Decision
Expected endpoint, settled by review	Proceed at tested parameters.
Minimal or no response	Reassess parameters with the medical director before increasing energy.
Prolonged erythema	Reduce parameters and re-test.
Blistering or crusting	Do not proceed. Medical director review. Follow SKN-009.
Any pigment change, light or dark	Do not proceed. Medical director review. Reconsider modality.

5 Quick Reference

QUICK REFERENCE CARD

Every new patient, every new device

1. **Score Fitzpatrick with the questionnaire.** Record in a fixed field.

2. **Ask about PIH after acne or injury** — it predicts better than the score.

3. **Check tanning status every single session.**

4. **Baseline photos:** practice device, fixed setup, no filters.

5. **Test patch at the intended parameters**, in a representative site.

6. **Wait the full interval.** Longer for Fitzpatrick IV–VI.

7. **Review in person and photograph.**

8. **Any pigment change at review → do not proceed.**

Print, laminate and post at the point of care

6 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

© MedSpa Standards. Licensed for use within the purchasing practice only. Redistribution or resale is prohibited.